

Psychiatric Ethics and The Law

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1000-1200

Live ZOOM

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Ethics and The Law Objectives

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At the end of this lecture, students will be able to:

- A. Identify the major medical ethical principles pertinent to medical/psychiatric care. (Including autonomy and Psychiatric Advance Directives)
- B. Paternalism
- C. The use of Power
- D. Understand what are the types of Boundary Violations
 - 1. Understand what is a therapeutic frame
 - 2. Understand what is a boundary crossing
 - 3. Recognize Red Flag Warnings of CA Medical Board
 - 4. Understand “General Truths”
- E. Discuss the essential aspects of confidentiality, and informed consent.
- F. Identify important factors pertaining to malpractice, including why psychiatrists are sued.

Ethics and The Law Objectives (continued)

At the end of this talk, students will be able to:

G. Understand The Tarasoff Decisions

H. Understand privilege and the various levels of privilege.

I. Discuss important CA civil commitment statutes. (e.g., 5150 and 5250)

J. Understand important legal cases in “Right to have”, and “Right to Refuse Treatment”.

K. Understand Penal Code Statutes and Supreme Court Cases especially regarding

1. Competency to Stand Trial (P.C. 1370) and
2. Not Guilty by Reason of Insanity (NGI: P.C. 1026) as well as the
3. M’Naghten Doctrine.

L. Understand the meaning of the legal terms: Mens Rea and Actus Rea

M. Understand what is meant by the term testamentary capacity.

Note: This lecture and or the video(s) that accompany it contain content that may elicit uncomfortable feelings in some students.

Vignette

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Mr. X., an 78 year-old man is admitted to a skilled nursing facility (SNF) after recently being treated for diabetes and congestive heart failure at a local hospital. He has evidence of a mild neurocognitive disorder. After being at the SNF for about 30 days he is convinced the food is being poisoned and that his roommate wants to hurt him. He threatens his internist, Dr. A., who sees him weekly, and says Dr. A. is an imposter. The family and doctor talk with Mr. X. about starting an antipsychotic medication, but he refuses.

The family secretly meets with Dr. A., and convince Dr. A. to covertly give Mr. X. an antipsychotic medication in liquid form to be sure he is not aware he is receiving it. In about a week, Mr. X. is doing much better. A few days later, a janitor tells Mr. X. he is taking antipsychotic medication and he immediately stops the medication and threatens to sue the doctor.

What ethical and legal issues are raised in this case?

1. Right to refuse medication.
2. Lack of informed consent for treatment.
3. Need for consultation**
4. Breach of Confidentiality (someone must have told the janitor)
5. Competency to agree to, or refuse medications
6. Consideration of involuntary psychiatric hospitalization
7. Assessing if an emergency exists
8. Problems in the therapeutic alliance? **
9. Lack of explanation of side effects of medications to Mr. X.

**Topics not fully discussed in this lecture

Important Ethical Issues In Clinical Practice

Six Major Ethical Principles

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▶ 1. Autonomy

- ▶ Capacity to be self-governing (self-determination)
- ▶ Based on
 - respect for persons,
 - **Voluntarism** (persons ability to make decisions they see as best for them, without coercion).
 - **Linked with privacy**: Right not to have mind or body intruded upon
- ▶ This approach emphasizes shared decision making

Other Major Ethical Principles/Terms

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▶ Autonomy Model:

▶ Benefits

▶ Changed Approach to Life Sustaining Treatment of the Dying Patient

▶ Made Informed Consent Much More Routine

▶ Shows **respect** for the patient

▶ Increased use of **Advance Directives**

Other Major Ethical Principles/Terms

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▶ Autonomy Weaknesses

▶ May not give enough substance to patient benefit

▶ Minimalism:

▶ In this type of Dr. / Patient relationship, doctor is overly concerned about not violating patient rights, thus could lead to:

▶ **Abdication** of health care decisions to the patient.

Ex: Oh, Mrs. Smith, you do not want to have xyz treatment which is 95% effective for your problem. You would rather have ABC treatment with virtually no hope for cure. OK., so be it. I will not try to dissuade you.

Major Ethical Issues

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- **Psychiatric Advance Directive (PAD) for Mental Health Care (began in 1991)**
- All states now authorize the execution of Advance Directives (AD) for mental health care.
- This growth complements shift from coercive model (CM) to a recovery-oriented model (ROM)
- Recovery Oriented Model (ROM):
 - stresses:** high-quality, evidence-based treatment, in a least restrictive setting.
 - reduces coercion:** attracts people into treatment by providing what they need and want, thereby reducing the necessity of pushing them into services .

Major Ethical Issues

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- **Psychiatric Advance Directive (PAD) for Mental Health Care** (continued)
- Surveys of mental health consumers show 66-77% would complete a PAD if given opportunity, or assistance.
- However: **only 4-13% have completed a PAD.**
- 2006 Study (Swanson) : Provided 1:1 assistance to consumers.
 - ▶ Results:
 - A. 83% meeting with facilitator filled out a PAD
 - B. Only 3% who did NOT meet with a facilitator filled out a PAD
 - C. **Facilitation increased treatment service satisfaction**
 - D. Facilitation improved working alliance with care givers

Major Ethical Issues

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Psychiatric Advance Directive (PAD) for Mental Health Care (continued)

Benefits :

- ▶ Allows families/proxies to speak directly with providers during crises.
- ▶ Allows families/proxies help make decisions during crises.
- ▶ Supports patient autonomy and empowerment in mental health care.
- ▶ May reduce involuntary treatment.
- ▶ May improve continuity of care.
- ▶ Honoring PADs is a Condition of Participation in Medicaid and Medicare for treatment facilities!

Exceptions for implementation:

- ▶ Does not meet community standards: I request pizza instead of medication for my treatment.
- ▶ Patient requests would interfere with treating an emergency
- ▶ Patient requests no involuntary treatment, but is suicidal and will not allow admission.
- ▶ Patient requests a specific hospital but a bed is not available.
- ▶ Conflicts with existing laws of the state.
- ▶ So: why is facilitation not used all the time? (\$\$\$\$\$)

Six Major Ethical Principles

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▶ 2. Beneficence

- Concerns patient benefit,
- Do good, do right by others
- ▶ Obligation to **promote welfare of others**
- ▶ Must take into account the effect it has for the recipient, not simply the initiator.
- ▶ **Beneficence untempered by autonomy was, and is often paternalistic;**
 - Paternalism relied too much on the wisdom and conscience of physician without sufficient regard for perspective and values of the patient
 - Ex: I. know what is best for you...why are you even bothering to question me?

Six Major Ethical Principles

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▶ 3. Nonmaleficence

- Term was coined in 1960s
- “First, do no harm.”
- Important consideration in therapeutic interventions that may cause adverse effects or outcomes
- **Examples:** In treating psychosis:
 - ▶ 1. Medications may cause TD
 - ▶ 2. Clozapine may cause agranulocytosis
 - ▶ 3. Medications may cause metabolic syndrome

Six Major Ethical Principles

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▶ 4. Justice

- A complicated ethical issue that requires consideration of fairness of a proposed treatment or intervention.

▶ Equitable or fair distribution of resources

- What obligations does a physician owe to a patient?
- What roles can providers have in providing or limiting resources to particular individuals?

▶ Typically decided at community public health level

▶ Examples:

- ▶ 1. Hospital organization will donate low/no-cost pediatric dental services to the community. There are only 45 openings per month. Justice requires a fair method, free from bias to determine who will receive the services.
- ▶ 2. Who on waiting list shall receive a kidney transplant
- ▶ 3. Determining number of vaccines to be distributed, and who shall receive them first. (Perfect timing in a pandemic!)

Six Major Ethical Principles

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▶ 5. Veracity/Truth Telling

- Patients expect physicians to tell them the truth.
- Some physicians find it hard to difficult to distinguish between obfuscation and compassionate provision of information.
 - ▶ Noted especially when dealing with patients and families from virtually all world cultures.
 - ▶ Belief that one can still practice veracity but at same time use “beneficent deception” in order to decrease patient anxiety.
 - ▶ Not all patients want to know the entire truth.
- **Raises issues of:**
 - ▶ When to explain to a family that a procedure went wrong?
 - ▶ When should one notify the Medical Board regarding a colleague who is impaired?

Six Major Ethical Principles

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▶ 6. Fidelity

- ▶ Requires **loyalty, fairness**, truthfulness, **advocacy**, and support and dedication **to one's patients**.
- ▶ It involves an agreement to **keep our promises**.
- ▶ Keeping a commitment ,based upon the virtue of caring.
 - ▶ Example:
 - ▶ A 78-year-old patient asks the doctor not to reveal that she is dying or give the diagnosis to her family. The patient says she does not want them informed because her family is very emotional and wants to do everything to keep her alive, even if it meant long-term mechanical ventilation. The patient has been very clear she does not want mechanical ventilation.
 - ▶ Keeping this information in confidence, while at the same time being supportive to the family involves fidelity

Other Major Ethical Principles/Terms

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▶ **Paternalism**

- ▶ The use of the psychiatrist's/physician's judgment about the best course of action for the patient.
 - ▶ Weak: Acting beneficently when the patient's impaired faculties prevent an autonomous choice.
 - ▶ Strong: Acting beneficently, despite the patient's intact autonomy
 - ▶ This can lead to negative feelings by a patient who feels her wishes are not being taken seriously.

Other Major Ethical Issues/Terms

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▶ **Paternalism** (continued)

- ▶ With paternalism, there is the potential conflict between what an individual would do or choose to do and what another person believes will benefit that person more. Thus, with paternalism, there is the creation of a:

- ▶ **Potential Conflict with Autonomy**

- ▶ Benefit (beneficence) vs. Autonomy is the conflict, as,
 - ▶ Paternalism emphasizes Beneficence

- ▶ Had been the routine for care delivery up to at least the 1960s.

Other Major Ethical Principles/Terms

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- **Paternalism** (continued)
 - ▶ With autonomy there is no reason to override patient wishes except in emergency
 - So, when patient is competent:
 - Durable Power of Attorney or
 - Advance Directive
 - ▶ Creates Surrogate Decision Maker

Other Major Ethical Principles/Terms

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But what happens if an Advance Directive does not exist?

Must go to court for judge to issue order to treat.

Note: in life threatening emergency no need to go to court.

Examples:

1. Patient was shot, is unconscious and is bleeding to death. No time to seek a court order to operate.
2. Patient fractures his leg. He refuses an operation which should be done to allow proper healing and avoid infection. He says he heard that poison vapors will be placed into his lungs by the anesthesiologist, so he has refused the treatment. This case needs immediate referral to a judge to assess competency to refuse the needed treatment.

Doctor Patient Relationship

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▶ **Sharing Power: A Complementary Model**

- ▶ Patients want power used in their behalf
- ▶ Doctor must use power carefully (not abuse it)
- ▶ **Power** consists of:
 - ▶ **Knowledge,**
 - ▶ **Technical Skill**
 - ▶ **Force of physician's personality**

Major Ethical Principles

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▶ ***The Ethical Use of Power***

Physician/Psychiatrist holds position of power compared to patient due to:

- ▶ Education
- ▶ Role as healer
- ▶ Socioeconomic standing
- ▶ Keeper of confidences
- ▶ Ability to involuntarily hospitalize patients
- ▶ Gatekeeper (medication prescriptions)

Major Ethical Principles

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- ▶ **Ethical Use of Power** (continued)
- ▶ **Psychiatric patients** have disorders that can impair their ability to:
 - ▶ Reason
 - ▶ Feel, and
 - ▶ Behave in an effective manner.
- ▶ Patients enter treatment at a time of vulnerability.
- ▶ Thus, **patients want power used in their behalf**

Major Ethical Principles

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- **Ethical Use of Power** (continued)
- Can you think of any situations in which **medical patients** enter treatment at a time of great vulnerability, and would want power used in their behalf?

Examples:

- Patient told he/she has cancer.
- Patient told he/she has severe heart disease.
- Patient told he/she needs a liver transplant
- Patient told he/she needs bypass surgery

Doctor Patient Relationship

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▶ *Sharing Power: A Complementary Model*

▶ The **Central Ethical Issue** in physician –patient relationship is:

▶ **The Responsible Use Of Power!!!!**

Major Ethical Principles

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- ▶ **Sharing Power: A Complementary Model**
- ▶ How do we develop shared power?
 - ▶ Through **Education** of the patient
 - ▶ This permits the patient to increase their power without weakening the physician's ability to fight disease/illness

Major Ethical Principles

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- ▶ **Sharing Power: A Complementary Model**
- ▶ A 75-year-old patient with atrial fibrillation says she read on the Internet that taking a blood thinner (i.e., Coumadin) can lead to a stroke. Her uncle died from a stroke and he was taking a blood thinner. So, she decided she will not take a blood thinner.
How would you proceed?
Can you force her to take the Coumadin?

A Model for Ethical Decision Making

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Clinical indications

How serious is the illness?
A need for Medical Intervention?
What is optimal standard of care?



Patient Preferences

What preference was expressed by the patient?
Is the patient capable of making this clinical decision?
What factors may be impinging on preferences of the patient?



Quality of Life (QOL)

What is patient's QOL given the illness process?
What impact will clinical intervention have on QOL?



External Considerations

Are external factors affecting patient care? (e.g., legal issues, limited programs)?

(Roberts 2004)



Doctor Patient Relationship

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Boundary Violations

A professional boundary is the “edge” or limit of appropriate behavior by a psychiatrist in the clinical setting.

A boundary violation is a boundary crossing that is **exploitative.**

Gratifies the doctor's need at the expense of the patient

•

Doctor Patient Relationship

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Boundary Violations

1. Non-Sexual Boundaries (Must not cross)

- ▶ Must Adhere to circumscribed length of appointments
- ▶ Must not accept (lavish) gifts
- ▶ Must Maintain Confidentiality
- ▶ Must not involve patients in social or business relationships
- ▶ Must not discuss the therapist's own problems
- ▶ Must see patients in a specific location, i.e., office or hospital
- ▶ Must have patient recognize that services are dispensed in a contractual context.
 - ▶ Must discuss fees openly

Doctor Patient Relationship

Boundary Violations (continued)

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Therapeutic frame:

The above boundaries when taken in toto, **plus**:

Being helpful,
being non-judgmental,
being understanding,
not being critical

Boundary Crossing:

A non-sexual boundary transgression in which the **ultimate effect is positive** in that it advances the therapy constructively

(Gabbard and Gutheil 1993)

Examples:

1. Child is getting chemotherapy. Stays in her bed all day long. Pediatrician offers the child \$5.00 for a gift if she gets out of bed. She gets out of bed and gets a gift for herself in the hospital gift shop.
2. Psychiatrist accepts a cookie from a VERY ill schizophrenic patient who has finally begun to trust the doctor.

A True Case

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A 36 year-old woman consulted a psychiatrist due to anxiety and disappointments in relationships with men. The psychiatrist felt he was simply a psychopharmacologist and ordered appropriate medication for her. He did not feel that psychotherapy would be beneficial for her as the medication would work much faster. He did not see any problem with an extraprofessional relationship that did not involve dating or physical intimacy. He agreed to her invitations to attend pool parties. He felt he was just being sociable, however, the patient felt more was involved and expected in the relationship.

Ethics In Psychiatry

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▶ 2. **Sexual Boundary Violations**

- ▶ For a psychiatrist to engage in a sexual relationship with a current patient is clearly not only **unethical**, but also...
 - ▶ **Legal sanctions** make such an ethical question moot
 - ▶ Statutes are now used to convict psychiatrists of:
 - ▶ Rape, sexual assault, sexual battery.
 - ▶ **Malpractice Claims:**
 - ▶ Most if not all insurance carriers will not insure against patient–therapist sexual relations and
 - ▶ Carriers specifically exclude liability for any such sexual activity

Ethics In Psychiatry

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▶ Sexual Boundary Violations (continued)

▶ Red Flag Situations:

- ▶ You start talking to the patient about the patient's personal life
- ▶ You check your personal appearance before a particular patient arrives
- ▶ A patient is scheduled at the end of the day to “allow more time”
- ▶ You allow your staff to go home early while you interact with a particular patient.
- ▶ You offer food or drink (to one but not all patients)
- ▶ You exchange gifts or hugs with a patient
- ▶ You offer free care to a particular patient
- ▶ You call the patient at home when the condition does not warrant it.
- ▶ You set up meetings with the patient outside the office.
 - ▶ Meet for coffee

Ethics in Psychiatry (CONTINUED)

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▶ But, is it unethical to:

- ▶ 1. Have **sex with a former patient?**
 - ▶ What if you ultimately married the patient?
 - ▶ What if 2 or more years pass before any involvement begins?
 - ▶ What if you only did a consult on a medical unit and never really treated the patient?
- ▶ 2. Have sexual activities with a patient's family member?
- ▶ 3. Give a hug to a patient when they disclose bad news (i.e., son just died) or trying to comfort a patient?
- ▶ 4. Receive Gifts or Services instead of money?
- ▶ 5. Accept a gift at the end of therapy?
- ▶ 6. Be friends with a patient on Facebook or Instagram?

Ethics in Psychiatry (CONTINUED)

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- ▶ According to the American Psychiatric Association (APA) Code of Ethics, a Sexual relationship is unethical!!!!
- ▶ (No matter what the circumstances)
- ▶ Note : American Psychological Association permits dating of former patients after waiting > 2 years.

Ethics

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► But, does this actually apply to an MD or DO, if they are not practicing psychiatry?

► Answer: YES!

► Here is the Proof

CA Medical Board and AMA Regulations

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- ▶ Physicians must be held to a higher standard than others, simply because of the very private nature of the doctor-patient relationship.
- ▶ There is a **"zero-tolerance"** as far as the courts in California, the Medical Board of CA, and the AMA are concerned.
- ▶ Any intimate or sexual relationship between a physician and a patient is **illegal and unethical**. There are no exceptions to this policy, nor should there be, **even in the case of mutual consent**.

CA Medical Board and AMA Regulations

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- ▶ According to the **AMA policy**, if an intimate relationship develops, physicians are expected to terminate the professional relationship at the least, although even this may not protect against allegations of unethical behavior if a physician "uses or exploits trust, knowledge, emotions or influence derived from the previous professional relationship."
- ▶ Penalty: Revocation or suspension of license, and civil lawsuits.
- ▶ **Best plan: Be extremely cautious.**
- ▶ **Dr. Z's plan...DO NOT DO IT**

Ethics in Psychiatry (Continued)

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- ▶ "General Truths" to identify known dynamics of sexual misconduct involving physicians:
- ▶ No matter how difficult or boundary testing the patient/client may be, It is ALWAYS the professional's responsibility to maintain appropriate boundaries or,
 - ▶ if unable to do so, to refer the patient/client for competent help or counsel.
- ▶ Sexual misconduct usually begins with relatively minor boundary violations.
 - ▶ Boundary violations include time, place/space, money, gift/services, clothing and language. It is a slippery slope.
- ▶ Crossing boundaries by a professional is almost always a power differential.

Ethics in Psychiatry (CONTINUED)

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- ▶ "**General Truths**" to identify known dynamics of sexual misconduct involving physicians. (continued)
 - ▶ The professional must refrain from obtaining personal gratification at the expense of the patient/client.
 - ▶ The main source of personal pleasure must come from the professional pleasure gained in helping the patient/client.
 - ▶ The **fee** for professional services is the only material satisfaction a physician should receive directly from the patient/client.
 - ▶ **No level of training**, nor school of medicine, school of law, school of dentistry, or school of psychotherapy **confers immunity** from sexual misconduct by a professional.

Ethics in Psychiatry (CONTINUED)

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- ▶ "General Truths" to identify known dynamics of sexual misconduct involving physicians.
(continued)
- ▶ consider this:
 - ▶ sexual misconduct cases will more often result in a temporary suspension of a license than any other type of case.
 - ▶ Sexual misconduct cases also are much more likely to result in a disciplinary outcome of surrender or revocation of license, versus probation. (For example, of 11 cases resolved during fiscal year 04-05, seven resulted in a surrender of license, three resulted in revocation and only one resulted in probationary terms and conditions.)

Ethics in Psychiatry (CONTINUED)

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▶ **The Seductive Patient:**

- ▶ Such patients display excessive admiration, affection and flattery
- ▶ Behaviors initially may seem benign to the clinician
- ▶ Why do patients act this way?
 - ▶ To assure safe supportive relationship
 - ▶ Such patients typically have psychological issues, ex. Victims of abuse, rape, incest or physical abuse

Ethics in Psychiatry (CONTINUED)

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- **The Seductive Patient (continued):**
 - ▶ Over involvement may occur with such patients if clinicians are:
 - Stressed
 - Depressed
 - Experienced illness or recent death in the family
 - Excessive work
 - Recent divorce, etc.

Ethics in Psychiatry (CONTINUED)

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- ▶ **The Seductive Patient (continued):**
 - ▶ Physician may have erotic feelings toward the patient
- ▶ **Remember:**
 - ▶ Feelings are O.K.
 - ▶ Problems arise when one acts on those feelings.

Ethics in Psychiatry (CONTINUED)

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- ▶ **The Seductive Patient (continued):**
 - ▶ OK, so even after all of this warning, what if you decide, to date a former patient? How can you try to minimize your risk?
 - ▶ Consult the Medical Board
 - ▶ Consider consulting an attorney
 - ▶ Be sure you have referred the patient
 - ▶ Wait > 1-2 years after referring the patient

Ethics in Psychiatry (CONTINUED)

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- ▶ **Seductive Patient (continued)**
 - **How should a clinician handle such patients?**
 - ▶ Communicate your role as physician
 - ▶ Discourage seductive behavior
 - ▶ Document seductive behavior in clinical record
 - ▶ Encourage patient to seek mental health support if indicated

Ethics in Psychiatry (CONTINUED)

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- ▶ Seductive Patient (continued)
 - How should clinician handle such patients(continued)?
 - ▶ Obtain consultation from a colleague if unclear how to handle the situation
 - ▶ Refer to another clinician
 - ▶ Never ignore seductive behavior
 - It may lead to legal issues

Ethics in Psychiatry (CONTINUED)

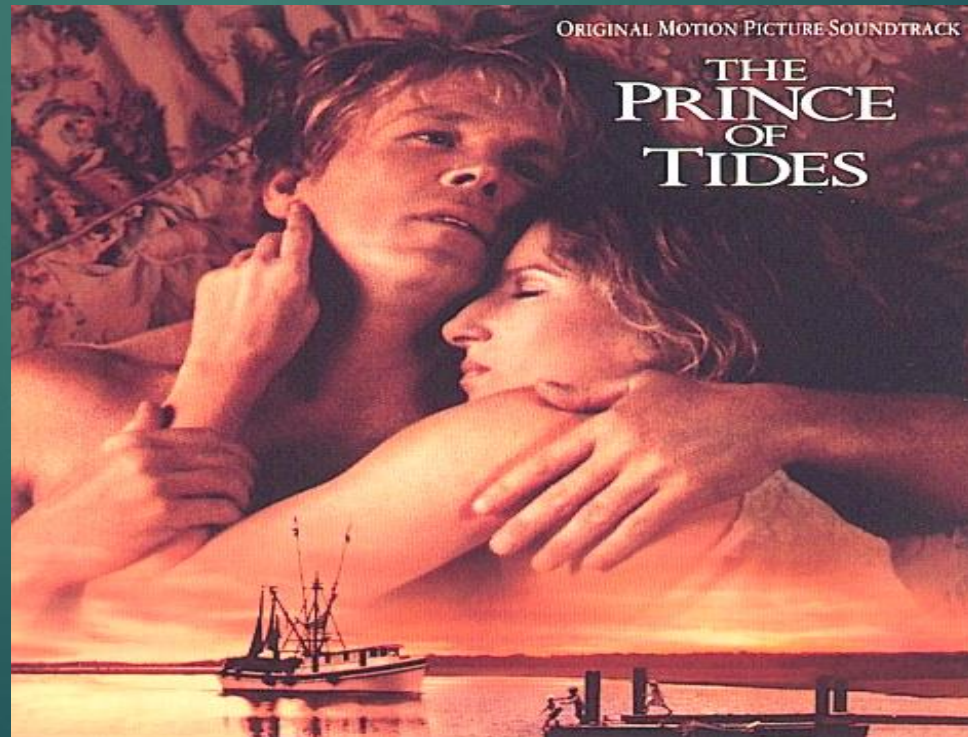
- ▶ Video (Time permitting)
 - ▶ Now that we all know about sexual misconduct, let us see Hollywood unfortunately portrays Psychiatry and sexual misconduct.

Ethics in Psychiatry (CONTINUED)

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- ▶ 1991 Barbra Streisand / Nick Nolte
- ▶ Let us see how the world famous New York Psychiatrist, Dr. Lowenstein shows us how **NOT** to deal with the issue of sexual boundary violations .

Prince of Tides
2.1



~2.5 min



▶ 3. Confidentiality

▶ “Whatever I shall see or hear in the course of my profession ... if it should not be published abroad, I will never divulge, holding such things to be holy secrets.

▶ Hippocratic Oath

▶ Who said the following:

▶ “Three people can keep a secret if two of them are dead”

▶ Benjamin Franklin, Poor Richard's Almanac

Doctor Patient Relationship

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▶ Confidentiality

- ▶ Refers to the therapist's responsibility not to release information learned in the course of treatment to third parties.
- ▶ To release information, one must have the permission of the patient.
 - ▶ Issue of privacy
 - ▶ Typically **continues after death**

Doctor Patient Relationship

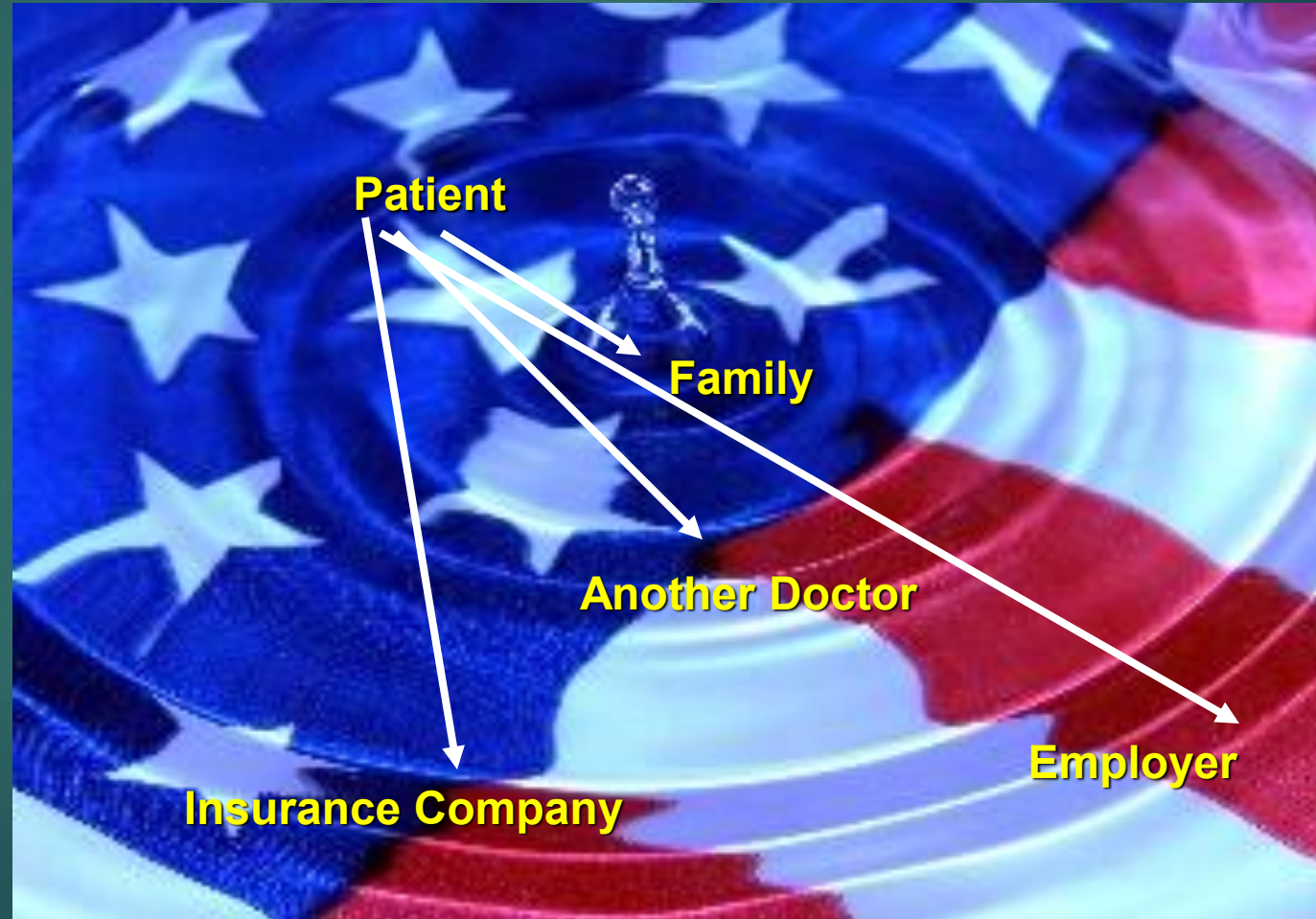
57

- ▶ **Confidentiality continued:**
- ▶ Think of confidentiality as being like **concentric circles**. The circles closest to center represent those people most likely to receive patient information. (ex: husband)
- ▶ Those several rings out are far less likely to receive such information: ex: employer
- ▶ **Remember:** Only the patient can decide how many circles from the center to allow information to be shared by the physician.

Concentric Circles of Confidentiality

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Only Patient
decides how
far out to
allow
information
to be shared



Doctor Patient Relationship

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▶ Confidentiality

▶ Confidentiality is very important

▶ Two risks if confidentiality broken

- ▶ 1. Information told to others may **Directly** harm the patient
- ▶ 2. **Indirectly** harms as now **patient likely reluctant to disclose additional information** for fear it will not be held in confidence.

Doctor Patient Relationship

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- ▶ **Exceptions to Confidentiality**

- ▶ **Subpoena**

- ▶ Does not automatically allow/require doctor to release the entire record
 - ▶ Doctor can ask for an in-camera (private) review (with a judge) to define what precise information must be disclosed.
 - ▶ Good idea to ask patient if OK to release this information.

Doctor Patient Relationship

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- ▶ **Exceptions to Confidentiality (continued)**
 - ▶ Reports to Courts
 - ▶ Reports to Insurance Companies
 - ▶ Be sure patient knows this is required!!
 - ▶ Workers Compensation Cases
 - ▶ High Risk to Others
 - ▶ Tarasoff Warnings
 - ▶ Impaired physician
 - ▶ Child / Elder Abuse
 - ▶ Suicidal / Homicidal Patient
 - ▶ Domestic Violence
 - ▶ Reportable infectious diseases (more usual with other specialties (HIV, TB, syphilis, gonorrhea))

This is Elder Abuse

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No Confidentiality here

Doctor Patient Relationship

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- Confidentiality

- ▶ The Internet

A non-psychiatrist physician was given a low score on an internet website by a patient.

He responded by pointing out on the website that the patient was psychotic at the time of the low rating.

Is this OK?

The patient reported the doctor to the state medical board for breaching confidentiality.

Doctor Patient Relationship

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Confidentiality

- ▶ A medical student is doing a rotation on an inpatient psychiatric unit. A patient confides to her that she has given away her belongings and will kill herself later that day. She cautions the student not to tell anyone as it will violate confidentiality.

The student is confused as to how to proceed.

What should she do?

Doctor Patient Relationship

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- ▶ Confidentiality
 - ▶ An employer calls a community psychiatrist to request information about a worker's mental health and addiction history. The employer tells the clinician that the company is entitled to the information because the company pays for the patient's insurance and the patient must be in good health to do the job.
 - ▶ What should the clinician do?

Informed Consent



Doctor Patient Relationship

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► Informed Consent

Justice Benjamin Cardozo (1914)

“Every human being of adult years and sound mind has a right to determine what shall be done with his own body; and a surgeon who performs an operation without his patient’s consent, commits a battery for which he is liable in damages”

What happened to Justice Cardozo in 1932?

Appointed to US Supreme Court by Herbert Hoover

Doctor Patient Relationship

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▶ Informed Consent

- ▶ The case in question: (Schloendorff v. New York Hospital 1914)

Woman gave consent to anesthesia with ether, so biopsy of fibroid tumor can be undertaken. Biopsy was found to be malignant and surgeon decided to go ahead and removed her ovary without her consent.

An infection, gangrene and the amputation of several fingers allegedly resulted from the operation. Hospital was sued, but not the doctor.

Doctor Patient Relationship

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- ▶ **Informed Consent (continued)**
 - ▶ Information must include:
 - ▶ Nature / risks of procedure / medications
 - ▶ Likelihood of bodily harm
 - ▶ Side effects
 - ▶ Probability of success
 - ▶ Alternative methods of treatment
 - ▶ Including no treatment

Doctor Patient Relationship

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- ▶ **Exceptions to informed consent:**
 - ▶ **A. Emergency** - Life Threatening Situations
 - ▶ **B. Patient does not possess capacity to make decisions**
 - ▶ **(Incompetent)** (**legal decision** determined by a superior court judge in CA)
 - ▶ **C. Waiver by the patient**
- ▶ **Key:** Be sure to document...then document some more!!

Doctor Patient Relationship

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- Assessing competency to give informed consent
- You do not need to be a specialist.
- We do it all the time when simply talking with a patient and noting if they seem to understand what we are saying.

- More formalized evaluation, 4 key points:
 - ▶ A. Do they **understand** the information being communicated?
 - ▶ B. Do they **appreciate** the nature of their situation, i.e., understand they are ill, and the impact of treatment?
 - ▶ C. Can they **reason** in some rational way about the situation, and weight risks and benefits of treatment?
 - ▶ D. Can they make a **decision**, or are they paralyzed by ambivalence?

Legal Issues

Malpractice

Legal Issues

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- Medical Malpractice

- ▶ Addresses harm caused by health care professionals.

- ▶ **Medical Malpractice Law does not expect Psychiatrists/physicians:**

- ▶ 1. To cure all patients

- ▶ 2. To perform at an-above average level

- ▶ **Medical Malpractice Law does expect Psychiatrists/Physicians :**

- ▶ 1. Will not intentionally (i.e., purposely) or

- ▶ 2. Will not negligently (i.e., unreasonably) harm their patients

▶ Malpractice: The 4 D'S of a Malpractice Claim

▶ Duty of Care: Doctor Agreed to Treat

- ▶ (in other words a doctor- patient relationship existed)
 - ▶ BTW, Can a physician refuse to see a patient?
 - ▶ In what situation (specialty) may a physician **NOT refuse** to see a patient?

▶ Deviation:

- ▶ Departure from accepted standard of care occurred (Did something...or did not do something)
 - ▶ Care was not up to standard of peers
 - ▶ (What the average clinician would have done.)

Legal Issues

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▶ Malpractice: The 4 D'S of a Malpractice Claim

▶ Direct Causation:

- ▶ Doctor's deviation directly caused the damage
- ▶ A. If doctor had not been negligent, would result still have been the same? (If yes doctor wins).
- ▶ B. Was the negligence the proximate cause? (If no, doctor wins)

▶ Damage (Harm) Occurred:

- ▶ There was some compensable damage to the patient. In other words the damage must be sizeable, not a mere inconvenience or scratch.
- ▶ Ex:
 - ▶ Doctor accidentally prescribes a lethal combination of medications. Patient loses the Rx., so no medication is taken. Thus, no damage occurred, no case.
 - ▶ Doctor orders Meperidine to a patient on an MAOI and patient develops Serotonin Syndrome and suffers anoxia due to an arrhythmia. (Damage occurred)

Legal Issues

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▶ Malpractice (continued):

Def.: Negligence in the conduct of one's professional duties.

- Approximately 2/3 physicians will experience a malpractice suit in their career.
- Reaction to threat of lawsuit is to **practice defensive medicine**, i.e., ordering excessive tests.
- Can a clinician be sued if he/she has never seen the patient?
 - **EX:** Get call from ER. You offer advice based on information supplied to you.

Legal Issues

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- Malpractice (continued):
 - Psychiatrists sued < than other physicians, because:
 - less likely to physically harm a patient
 - Patient is reluctant to disclose mental disorders/treatments
 - Typically no witnesses to alleged malpractice
 - Psychiatrists tend to have long term relationships caring for their patients which may decrease likelihood of a lawsuit.

Legal Issues

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- Malpractice (Continued):

Why are psychiatrists sued?

1. **Suicide/Attempts**, especially in a hospital (17%)

- a. May have been reason for hospitalization

- b. Errors

- ☐ Failure to get adequate history of suicidal behavior.
- ☐ Failure to adequately protect patient (1:1)
- ☐ Failure to communicate changes to doctors/nurses

2. **Wrong Treatment** (36%)

3. **Incorrect Diagnosis** (8%)

Legal Issues

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- Malpractice (Continued):

4. Failure to obtain **informed consent** (4%)
5. **Medication error or drug reactions** (14%) i.e., T. D. diabetes
6. **ECT** claims (informed consent lacking) or not giving ECT (1%)
7. **Abandonment**: No time for patient to find another doctor. (3%)
 - a. What about a patient who threatens a clinician?
 - b. What about a patient who does not pay?
 - c. Coverage during absences?
8. **Undue Familiarity with** a patient (3%)
9. Unnecessary **Commitment** (4%)
- 10 Breaching **Confidentiality** 3%

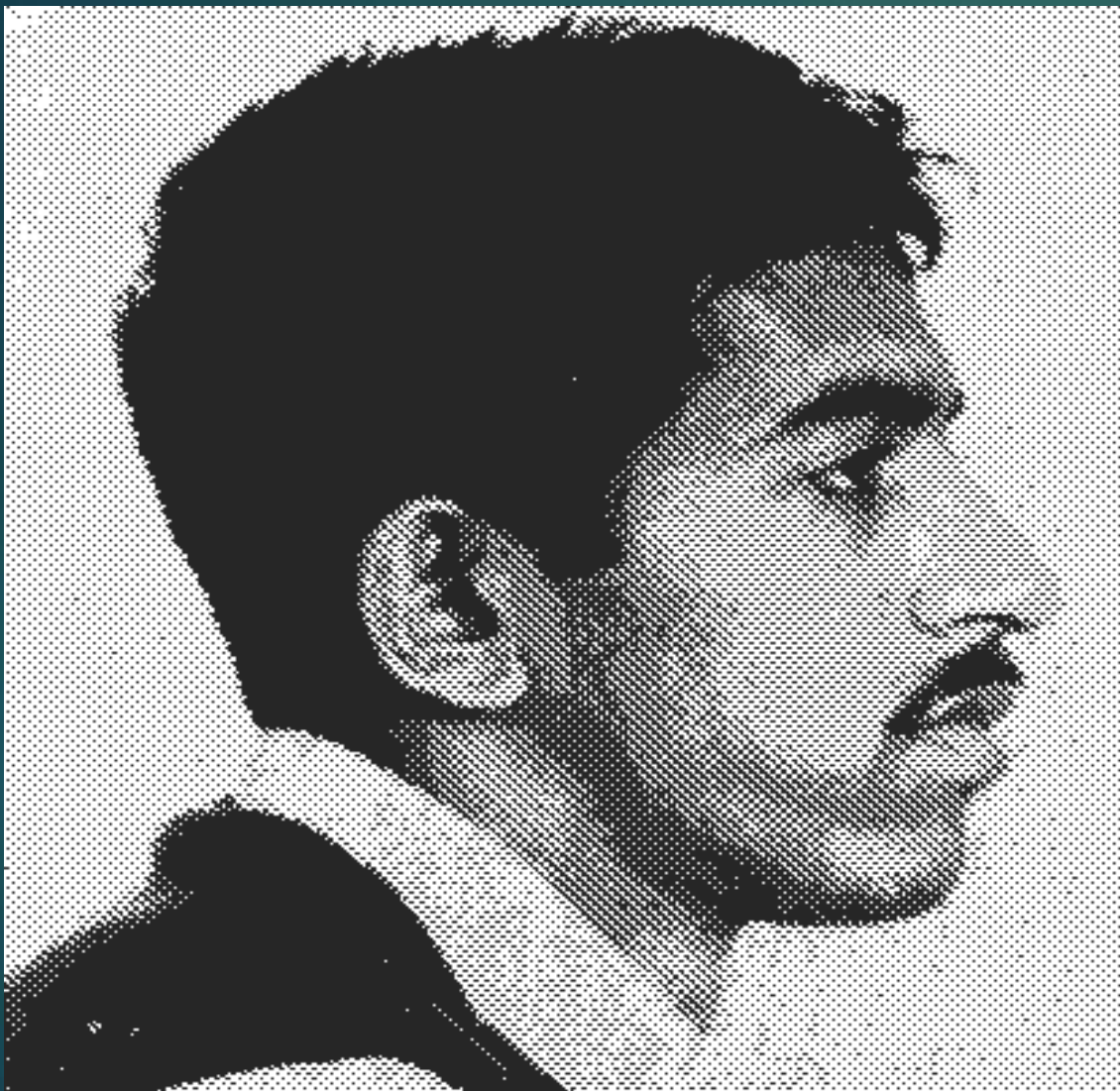
The Key Court Decision Concerning:

The Violent Psychiatric Patient

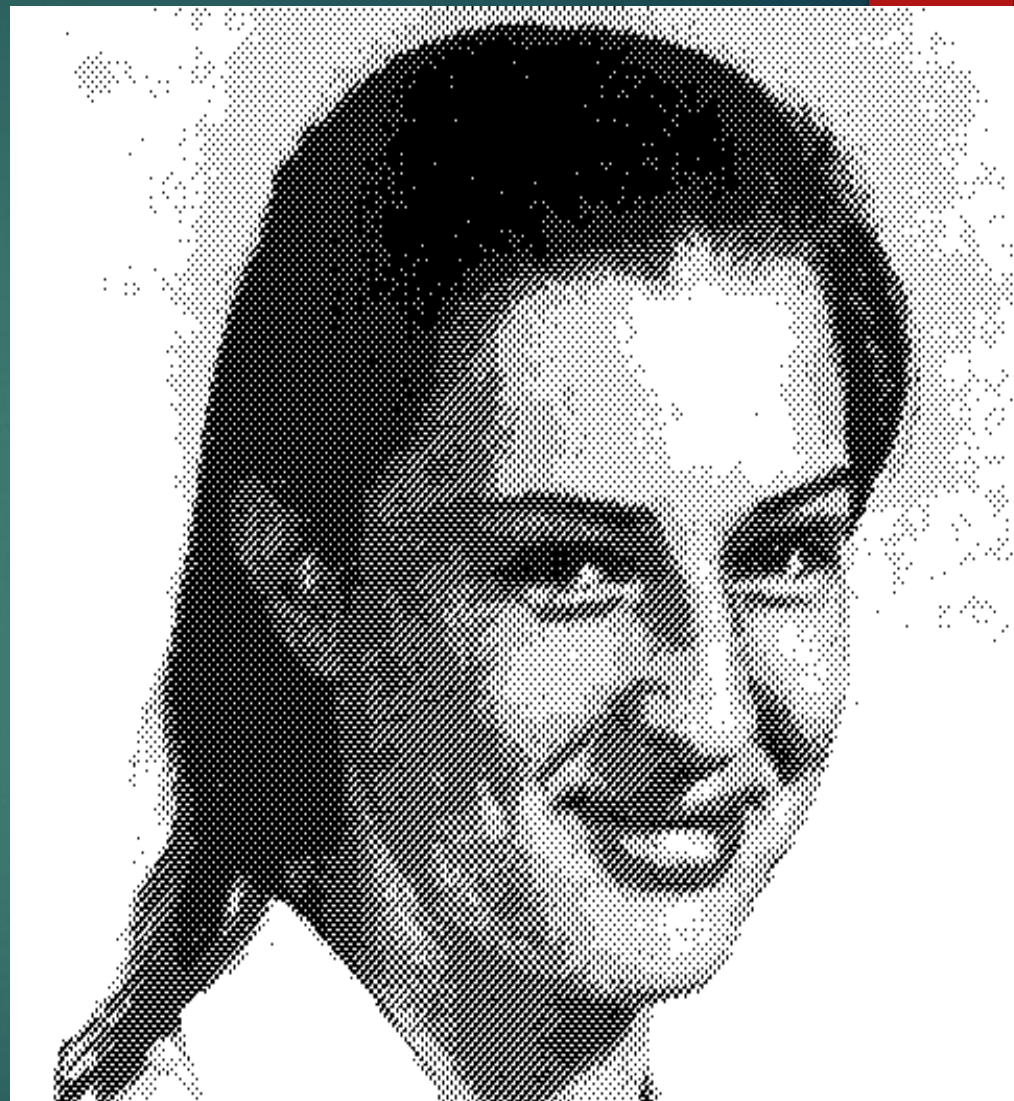
► Tarasoff v. Regents of the University of California (1976) (Tarasoff I)

1. Therapist knew of danger to Tarasoff and wanted to involuntarily admit Poddar for psychiatric observation / treatment for 72 hours (5150)
2. Actually notified UC Berkeley campus police orally and in writing about danger of Poddar and that he should be committed.
3. Campus police detained Poddar, but let him go after he agreed to “leave her alone.”
4. He murdered her after her vacation.

PODDAR



Tatiana Tarasoff



Legal Issues

- ▶ **Tarasoff v. Regents of the University of California (1976)**
(Tarasoff I) (continued)

- 4. **Ruling by CA Supreme Court (> 14 months of deliberation)**
 - ▶ **Duty to Warn** potential victim if therapist feels patient may injure or kill someone that is identifiable.
 - ▶ To meet this duty, therapist must:
 - ▶ Notify intended victim, or others likely to notify victim of the danger.
 - ▶ Notify the police

- ▶ **Imminent Threat...but---No Identifiable Victim...
cannot issue a warning**

Legal Issues

Tarasoff v. Regents of the University of California (1982) (Tarasoff II)

86

Broadened the earlier ruling, extended duty to **warn** to now emphasize a duty to protect!

1. How does one protect the potential victim?

- Call police
- Increase or start medications as warranted
- Involuntarily commit the patient
- Assess if danger is imminent and identifiable victim is in great danger.
- Bring in family members as needed
- Assure guns removed from home

- **What is Privilege?**

Legal Issues

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- ▶ **Privileged Information (Privilege)**
 - ▶ A Legal Concept
 - ▶ Pertains To Judicial Proceedings
 - ▶ Belongs to the Patient
 - ▶ May Only be **waived by the patient**
 - ▶ When can one testify about a patient?
 - ▶ Waiver by patient / attorney
 - ▶ Ordered by the Court
 - ▶ You may refuse to testify, but may be held in contempt of court

Legal Issues

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Levels of Privilege (low to high)

- Doctor – Patient (lowest)
- Psychiatrist – Patient
- Attorney – Client
- Clergy – Client (Highest)

Note: The greater or higher the privilege allows individual to usually avoid testifying, as judge will refuse to order such testimony. The above list is from least privilege on top to greatest privilege on the bottom. This explains why, for example, one can speak freely to their attorney without fear that such discussion will become known at trial.

Who is he?

90



Ordained Minister



Who is he?

What did he do?

91

Sirhan Sirhan

Assassinated
RFK in 1968



Recommended for parole: 8/21. It was denied by Gov. Gavin Newsome. Has now been in prison 55 years. He is 80 years old. Debate over his release continues. He had a hearing in 3/23 and was denied parole. Next opportunity for parole is in 2026.



What is his relationship to R. Grier?

Civil Commitment Statutes

Legal Issues

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Civil Commitment Statutes

- ▶ **Grave Disability (5150)** a 72 hour hold
 - ▶ **Due to a Mental Disorder individual is:** Unable to Provide for:
 - ▶ **Food, Clothing or Shelter. (Only one of 3 needed)**
 - ▶ **May also be held on a 5150 as:** **Danger to Self or Others**
- ▶ **Note:** Patient cannot legally contest being held involuntarily during this period.
- ▶ Patient can be discharged at any time but only if doctor agrees to the discharge.

Legal Issues

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Civil Commitment Statutes

- ▶ Grave Disability or danger to self /others (5250)
 - Permits up to 14 days of additional involuntary commitment after 5150 expires.
 - Patient can now contest the involuntary hold.

Legal Issues

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Involuntary hospitalization is an ethically complex area

- ▶ **Beneficence:** Doing good in patient's best interest
- ▶ **Nonmaleficence:** Physicians should not engage in actions that harm patients
- ▶ **Autonomy:** Respecting the patients capacity to make decisions
- ▶ **Justice:** Treat patients equally and medical resources should be distributed freely.

Burdens of Persuasion

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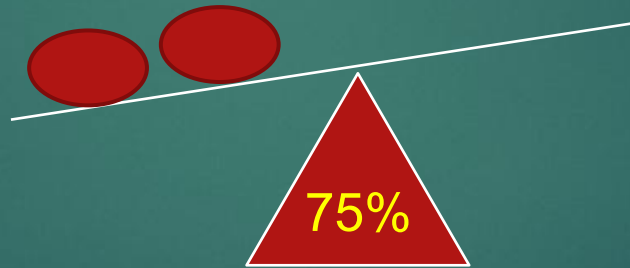
Preponderance of the evidence:

1. Civil litigations (Judge Judy)
2. **Competency to stand trial (1370)**



Clear and Convincing evidence (Proof is more likely than not):

1. **Involuntary commitment**
2. **Forced medications**
3. Guardianship



Evidence beyond a reasonable doubt (Little to no doubt.)

1. Criminal cases



Important Court Decisions Regarding Involuntary Psychiatric Treatment

The Right to Treatment

Legal Cases

99

► *Important Right To Treatment Decisions*

1966: Rouse v. Cameron (St. Eliz. Hosp.) (NGI)

Was not receiving psychiatric treatment. He was released for lack of treatment.

- Should be released to lesser supervision whenever possible.

Legal Cases

100

- ▶ **Important Right To Treatment Decisions**
- ▶ **1971: Alabama Fed. Court (Wyatt v. Stickney)**
 - ▶ Civilly committed must be **treated**.
 - ▶ Set minimum **staffing** ratios
 - ▶ **Nutritional** standards, etc.
 - ▶ **Violated 14th Amendment** rights
 - ▶ no state could “deprive any person of life, liberty, or property, without due process of law; nor deny to anyone in its jurisdiction the equal protection of the laws due process of law; nor deny to any person within its jurisdiction.

Legal Issues

101

▶ *Important Right To Treatment Decisions* (continued)

▶ 1976: O' Connor v. Donaldson (US Supreme Court)

- ▶ Finding of mental illness not sufficient to confine patient against their will.

- ▶ Must instead also be considered

- ▶ dangerous to self or

- ▶ Danger to others or

- ▶ unable to care for themselves so cannot survive outside the hospital.



The Right to Refuse Treatment

LEGAL ISSUES

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▶ *The Right To Refuse Treatment (Medication)*

▶ **1979** Rogers v. Okin. Mass. Case

- ▶ No seclusion or restraint unless an emergency exists.
- ▶ No medication for individuals without their consent.
- ▶ Case went to Mass. Court of Appeals

- ▶ if patient was competent they could decide on their own.

- ▶ If state felt individual was incompetent to make such a decision, then the superior court had to decide if such was the case... i.e., was the patient incompetent to refuse medication

LEGAL ISSUES

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▶ *The Right To Refuse Treatment (Medication)*

▶ **1981 Rennie V. Klein (New Jersey case)**

- ▶ Decided by 3rd circuit court of appeals.
 - ▶ Issue was whether compulsory medication violated a liberty interest protected by the 14 amendment and if such an interest exists, then
 - ▶ What procedures the state should follow to protect it.
 - ▶ Set up decisions outside of superior courts (as was instituted in Mass.) Going to court is very time consuming and cumbersome.
- ▶ Note: In emergency always can force medications.
But must be sure to document reason(s) for doing so!!

Penal Code Statutes

Incompetent to Stand Trial (P.C. 1370)

Competency to Stand Trial (P.C. 1370)

▶ US Supreme Court Case:

- ▶ Dusky v. United States, 1960
- ▶ Two Basic Tests
 1. Ability to **Cooperate** with defense attorney
 2. Able to **Understand** the proceedings / charges and consequences of conviction

Competency to Stand Trial (P.C. 1368)

- ▶ If there is a question raised regarding the defendant's competency to stand trial a psychiatrist or psychologist is assigned by the court to evaluate the defendant. Usually one is selected for each side.
- ▶ At this point the court proceedings will be halted and a forensic evaluation will occur (PC1368).
- ▶ The evaluators will supply the court with their findings within a specified period of time, i.e., 2 weeks.

Evaluation of Competency to Stand Trial (P.C. 1368)

What do evaluators assess for?

1. Ability to relate to the attorney
2. Ability to work with their attorney to plan legal strategy including a guilty plea to lesser charges if applicable
3. Ability to understand roles of those involved in the trial. (i.e., what does the judge do, what is a jury, role of the attorneys for each side, etc.)
4. Understanding of court procedure
5. Appreciation of charges.
6. Appreciation of range and nature of possible penalties
7. Appraisal of the likely outcome of the trial
8. Capacity to disclose to attorney available pertinent facts surrounding the offense

Evaluation of Competency to Stand Trial (P.C. 1368)

What happens after the forensic report(s) are received?

1. If found by outside evaluators to be competent to stand trial, the trial will again proceed.
2. If found incompetent to proceed, the trial is halted and the individual is placed on a PC 1370 hold and sent to a CA State Hospital for treatment. (Napa State Hospital for most Northern CA individuals) In a few cases of minor offenses the defendant may be allowed to be treated as an outpatient.
3. Once under treatment reports to the court must occur approximately every 90 days outlining the defendants ability to return to court.

Legal Issues

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How long can someone be detained as incompetent to stand trial(P.C. 1370)

- US Supreme Court Case:
 - ▶ **Jackson v. Indiana, 1972**
- Court said all efforts to restore sanity must occur. Cannot be held more than the reasonable period of time necessary to determine whether there is a substantial probability that he (or she) will attain that capacity in the foreseeable future.
- In CA three years are permitted to regain competency.
- Treating physicians must notify the court as soon as they reach the conclusion the individual either has been restored to sanity or will never be restored to competency. No need to wait any specified period!

Legal Issues

112

How long can someone be detained as incompetent to stand trial (P.C. 1370) (continued)

- **US Supreme Court Case: Jackson v. Indiana, 1972**
- If it is determined that the individual will never be restored to sanity, the State must either institute civil commitment proceedings that would be required to commit indefinitely, or release the defendant.

Example: 79 year-old man sent to NSH for treatment on a 1370 hold. He shot his gun into the walls of his home believing people were hiding there and were stealing his clothing. Unfortunately, a bullet went through the walls and hit his next door neighbor while he was asleep in his bedroom. He later died. After a few weeks it is clear that due to his Alzheimer's dementia that he will never improve and there is no chance he can be restored to sanity. He is sent back to the court for disposition. In this case he likely would be placed back in his home in the county, under the auspices of a conservator and placed in an assisted living facility.

He was found Incompetent to Stand Trial. Who is he?

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Shot
Gabrielle
Giffords 1/11



Jared
Loughner

After being
found
competent
to proceed,
agreed to
life without
parole in
Federal
Prison
11/12.

She was found incompetent to stand trial. Who is she?

114

Alexandra
Souverneva

Allegedly set
Fawn fire in
Redding, CA in
9/2021.

At preliminary
hearing judge
declared she
should stand trial
for felony arson



10/21: Court
declares her
incompetent to
stand trial. Sent for
treatment to regain
competency.

Declared
competent to stand
trial in 2022.

June 2023
Pleads not
guilty

12/24:
Trial has still not begun
but date set for 5/25
9/25:
No new information
available on what is
occurring in the case.



Not Guilty By Reason Of Insanity (P.C. 1026)

▶ The Insanity Defense (P.C.1026)

1. M' Naghten Doctrine 1843:

- ▶ From British law (known as the right/wrong test)
- ▶ Persons are not guilty by reason of insanity if they labored under a mental disease such that they were unaware of the **nature, quality and consequences** of their **acts**, or if they were **incapable of realizing** that their **acts were wrong**.
- ▶ This was the rule used in the USA for long time. (right wrong test)
- ▶ (**Currently** the **American Law Institute** (ALI) Model Penal Code is used)
 - ▶ Person with mental disease or defect is not held criminally responsible **if**:
 - ▶ Could not appreciate wrongfulness of the act
 - ▶ Could not conform the conduct to the requirements of the law.

NGRI Statistics

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- ▶ Isn't it true that it is easy to raise the insanity defense and then be found NGRI?
- ▶ Facts
 - ▶ Only 0.85%— actually raise the insanity defense nationwide.
 - ▶ Percentage of all defendants found NGRI is around 0.26%

Who are They?

Why are they waving?



What is the circumstance?

Who is This Man?

What relationship to NGI does he have?

120

His attorney plans to ask the court, by the end of this year, to have his client totally released from judicial oversight.



John Hinckley Jr.

Shot President Reagan and Jim Brady in 1981

Found NGI after Shooting

8/18: Appealing to court for unconditional leave.

9/16: Placed on Home leave 35 years after shooting.

2021: Court ruled he will be totally free in 2022. He is now free.



▶ Prison vs. State Hospital

- ▶ Does it really matter if a seriously mentally ill person is sent to prison, as Guilty, or to a State psychiatric Hospital as NGI?

What are the major differences for the patient?

- ▶ **Treatment**: Much more definitive treatment in a state psychiatric hospital. That is their mission
- ▶ **Release**: Release under supervision but, ONLY after the individual has been found to no longer present a danger to himself or others.
 - ▶ **Important point**: Most individuals sent to prison for a crime will spend less time in prison than if sent to a psychiatric hospital as not guilty by reason of insanity (PC1026).
- ▶ Ex: Man who is psychotic steals a car and wrecks it. If convicted he might serve 2 years in a State Prison.(1 year with good behavior) If found NGI, likely might serve >2 years if his symptoms cannot be controlled. So, his attorney might recommend he not plead NGI.

San Quentin Prison Cell



Napa State Hospital Dorm Room

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Food For Thought

123

► Treatment of Mentally ill in CA 2009-2019

2009: 80,000 in jail custody in CA.

15,500 had active MH cases (19%)

10,500 were receiving psychotropic medications (13%)

2019: 72,000 in jail custody in CA.

22,000 had active MH case (30.5%) (11.5 % increase)

19,000 were receiving psychotropic medications (26.3%)

By comparison, California's five state psychiatric hospitals combined now serve fewer than 6,000 patients.

More Food for Thought

124

Individuals experiencing mental illness are likely to remain incarcerated longer than their peers without a mental illness

People with a mental illness are more likely to be disciplined and isolated in segregated housing (i.e., solitary confinement).

Solitary confinement conditions worsen the symptoms of mental illness.

Moreover, they often find it exceedingly difficult to meet the requirements for release.

People with a mental illness diagnosis are more likely to commit suicide and/or be victimized. Suicide is the leading cause of death in correctional facilities, and as many as half of all inmate suicides are committed by the estimated 15 to 25 percent of inmates with serious mental illness.

Untreated mental illness can also contribute to violent victimization in custody.

Individuals with serious mental illness in jail were five times more likely to report that they were sexually victimized by another inmate than individuals with no mental illness.

It is expensive to incarcerate individuals with mental illnesses since jails in the United States are improperly equipped for treatment.

Correctional health care professionals are constantly constrained by limited or improper resources and large caseloads.

Community mental health treatment is less costly and more effective than incarceration.

Important terms:

1. Mens rea: Criminal intent (guilty mind)

- ▶ In essence ,what was the defendants mental state and what did the defendant intend when the crime was committed. Permits courts to differentiate between a person who had not intended to commit a crime and one who did. (Society does not want to punish someone who innocently caused harm).
- ▶ Examples:
- ▶ **Driver A.** sees a pedestrian dart into the roadway. He slams on his brakes. He still hits and kills the pedestrian. **Driver A.** likely will not charged criminally, but may be sued in civil court monetarily. (Why you need insurance!)
- ▶ **Driver B.** Sees a pedestrian dart into the roadway. He does not brake, instead he hits the gas pedal and purposely runs over the pedestrian, killing him. **Driver B.** is probably criminally liable because he intended to kill the pedestrian, or at least he intended to cause serious bodily harm.

Important terms (continued) :

2. Actus reus: A criminal (guilty) Act:

Requires an overt act in furtherance of a crime. This means society has chosen to punish only bad deeds, not bad thoughts.

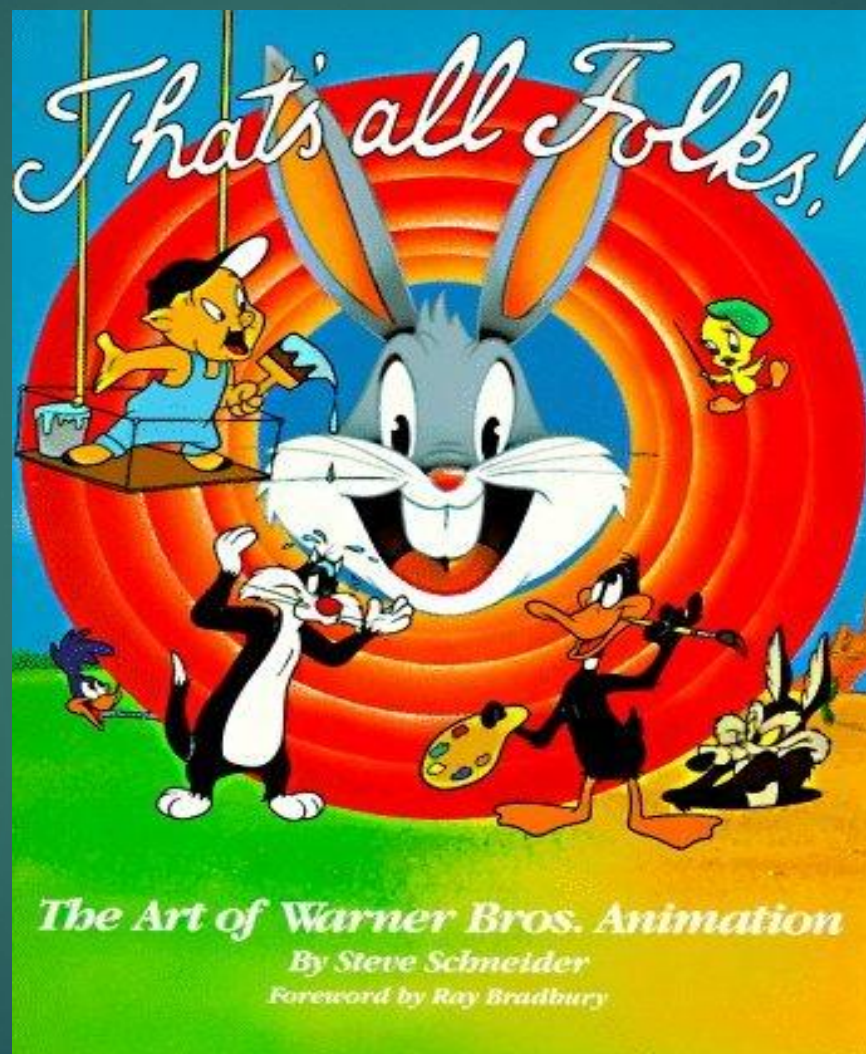
Example:

1. Young man has thoughts of stealing a Corvette and driving it in a race. He never steals the car. No charges.
2. Young man has thoughts of stealing a Corvette and driving it in a race. He acts on the thoughts and steals the car. He enters a race and **wins**. He is charged with a felony, and **loses** his freedom.

Testamentary Capacity

- Comprehends Nature and Extent of Property
- Understands the Natural Objects of Bounty
 - (Which heirs will get your property)
- Understands Nature of One's Acts
 - (What writing and signing your will means.)

Note: remember that testamentary capacity refers to **wills and **not testifying!****



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